

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in New Jersey

A registered & accredited Health Care Service Firm — private pay & Medicaid (MLTSS).

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New Jersey model — read this first

In New Jersey, a business that sends caregivers into people's homes for personal care and companionship is a Health Care Service Firm (HCSF) — and it registers with the New Jersey Division of Consumer Affairs, NOT the health department, under the Private Employment Agency Act (N.J.S.A. 34:8-43 et seq.) and rules at N.J.A.C. 13:45B. Two things make New Jersey distinctive and stricter than most states: your agency must be ACCREDITED as a condition of registration, and your hands-on caregivers must be Certified Homemaker–Home Health Aides (CHHAs) certified by the New Jersey Board of Nursing. This guide walks the whole path — accrediting, registering, staffing, and getting paid — broken down so someone who has never done any of it can follow it.

Two New Jersey requirements you won't find in most states. (1) Accreditation is MANDATORY — you must be accredited by an approved body (CHAP, ACHC, or The Joint Commission; NJ also recognizes CAHC and NIHCA) to register and renew as an HCSF. Start it early — it's a precondition, not a nice-to-have. (2) Certified aides — a caregiver who provides hands-on personal care must hold a CHHA certificate from the NJ Board of Nursing (76 hours of training plus a fingerprint background check). Plan your timeline and budget around both.

You also post a bond. New Jersey requires HCSFs to carry a \$10,000 surety bond (waivable if your net worth is at least \$100,000). It's inexpensive (often around \$100/year) but it's a registration requirement, so line it up with your insurance.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened an agency — every time we name a thing, we stop and break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with the Division of Consumer Affairs and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a registered, accredited, paid agency.

1	Decide this is your business	A registered, accredited home-care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance & the \$10,000 bond (Part 3)
3	Get accredited	CHAP, ACHC, or TJC — REQUIRED, so start early (Part 4)
4	Register as an HCSF	Consumer Affairs registration, bond, background checks (Part 4)
5	Hire CHHA-certified aides	The NJ Board of Nursing CHHA certificate (Part 5)
6	Set up EVV & operations	HHAEExchange EVV, policies, systems (Part 6)
7	Serve private-pay clients	Start earning while Medicaid

		contracting proceeds (Part 8)
8	Contract for Medicaid (MLTSS)	NJMMIS enrollment + the 5 managed-care plans (Part 8)
9	Get your first clients	Discharge planners, the 21 county AAAs, ADRC (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Accreditation is the long pole — it can take months and you can't register without it, so start writing policies to the accreditor's standard on day one. Line up your \$10,000 bond and your CHHA hiring pipeline in parallel, and launch on private pay while Medicaid contracting proceeds.

PART 1

What This Business Is

In short: A registered, accredited Health Care Service Firm — you employ and place Certified Homemaker–Home Health Aides to help clients with daily living at home. Non-medical, agency-based, private pay and Medicaid.

New Jersey regulates agencies that employ or place caregivers in clients' homes as Health Care Service Firms. Your firm provides non-medical care — help with the activities of daily living (bathing, dressing, grooming, toileting, transferring, eating, mobility) plus homemaker tasks (meals, housekeeping, laundry, errands) and companionship. Because you employ and place the aides (rather than the client hiring them directly), you're an HCSF and must register — whether you serve private pay or Medicaid.

Two regulators, three things they govern

What	Who governs it	The rule
The FIRM (HCSF registration + accreditation)	Division of Consumer Affairs	N.J.S.A. 34:8-43 et seq.; N.J.A.C. 13:45B
The AIDE (CHHA certification)	NJ Board of Nursing	N.J.S.A. 45:11-24.3
The BOND	Division of Consumer Affairs	N.J.A.C. 13:45B-13.3 (\$10,000)

This guide is the non-medical HCSF. If you ever add skilled nursing, that's a separate Home Health Agency licensure through the Department of Health — a different, heavier path.

Key terms, in plain English. HCSF = Health Care Service Firm (your registration). DCA = Division of Consumer Affairs (your regulator). CHHA = Certified Homemaker–Home Health Aide (your caregivers). MLTSS = Managed Long Term Services and Supports (Medicaid home care). PCA = the Personal Care Assistant benefit. PPP = Personal Preference Program (the consumer-directed option). MCO = a NJ FamilyCare managed-care plan. NJMMIS = NJ Medicaid's provider system. EVV = Electronic Visit Verification (HHAeXchange). ADRC = Aging & Disability Resource Connection. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. New Jersey is one of the highest-cost, highest-density home-care markets in the country — which means strong private-pay rates and a deep pool of families who need and can arrange care. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: *Consumer Affairs registers and accredits your firm and holds your bond, the Board of Nursing certifies your aides, NJ FamilyCare/DMAHS and the MCOs pay you for Medicaid, and HHAeXchange verifies Medicaid visits.*

Division of Consumer Affairs — Regulated Business Section (your registrar)

The DCA's Regulated Business Section registers your HCSF, confirms your accreditation and \$10,000 bond, and processes owner/officer background certifications. Registration renews every July 1. Phone (973) 504-6370; njconsumeraffairs.gov/hcservice.

NJ Board of Nursing — CHHA certification

The Board of Nursing (within DCA) certifies your hands-on caregivers as CHHAs and maintains the CHHA registry you verify before assigning an aide. Phone 973-504-6430; njconsumeraffairs.gov/hhh.

NJ FamilyCare / DMAHS and the managed-care plans (Medicaid)

NJ Medicaid (NJ FamilyCare), through the Division of Medical Assistance and Health Services, delivers home care via Managed Long Term Services and Supports (MLTSS) and the Personal Care Assistant benefit — both through the managed-care organizations. You enroll via NJMMIS and contract with each plan. ADRC statewide line 1-877-222-3737.

EVV — HHAeXchange

For Medicaid personal-care visits, New Jersey uses HHAeXchange as the statewide EVV aggregator (some plans route through CareBridge into HHAeXchange). A claim must match a verified visit to be paid. Private-pay work isn't subject to EVV.

The Long-Term Care Ombudsman & Adult Protective Services

The NJ Long-Term Care Ombudsman advocates for people receiving long-term care, and county Adult Protective Services takes abuse/neglect reports — you are a mandated reporter. Build both into your policies and caregiver training.

What this means for you: five different bodies touch your business — Consumer Affairs (the firm), the Board of Nursing (your aides), your accreditor, NJ FamilyCare and the MCOs (your Medicaid pay), and the ombudsman/APS (resident protection). It sounds like a lot, but each has a narrow job, and this guide tells you exactly who to call for what (Part 11). Keep a simple contact sheet on your wall from day one.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New Jersey you form the LLC through the Division of Revenue & Enterprise Services, then complete the NJ Business Registration (NJ-REG) that gives you your tax and employer IDs. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Certificate of Formation.** Through the Division of Revenue at nj.gov/treasury/revenue — the filing fee is \$125.
- 2. Complete the NJ-REG business registration.** Required after you form the entity, to get your NJ tax and employer identification and to hire employees.
- 3. Name a registered agent and adopt an Operating Agreement.** The agent (which can be you) receives legal mail at a NJ address; the Operating Agreement sets ownership and management.
- 4. Calendar the annual report.** NJ LLCs file an annual report — \$75 — each year with the Division of Revenue to stay in good standing.

No newspaper publication in New Jersey. *Unlike New York, NJ has no LLC publication requirement — forming the entity is a straightforward filing plus the NJ-REG registration.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with NJ Medicaid and bill the MLTSS plans. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of NJ branches — TD Bank, Bank of America, Wells Fargo, PNC, or a NJ option like Provident or Columbia, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance & bonding — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so you need coverage built for that risk — and New Jersey requires the \$10,000 surety bond as a condition of registration:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (an aide's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by an aide — general liability EXCLUDES this	Yes — high exposure
\$10,000 Surety Bond	Required by the HCSF Act to register (waivable at \$100k net worth)	Yes — required to register
Non-Owned & Hired Auto	Your aides driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation & Disability	Aide injuries; NJ also mandates temporary disability coverage	Required in NJ for employees
Employee Dishonesty Bond	Theft from a client's home — families ask if caregivers are “bonded”	Strongly recommended

New Jersey requires disability coverage too. On top of workers' comp, NJ employers must carry Temporary Disability Insurance for employees. Your payroll provider or broker sets this up — factor it into your labor cost.

Real places to get it

Buy through an independent agent or broker who writes New Jersey home-care programs — they can also place your \$10,000 HCSF bond. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local NJ agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care), or ask a franchise or peer. Confirm the carrier writes non-medical home care in New Jersey before you rely on a quote.

What to ask the insurance agent

“Do you write NJ Health Care Service Firms — which carriers, and what's the minimum premium?”

“Can you place the \$10,000 HCSF surety bond, and is abuse & molestation included or a sub-limit?”

“Do you include non-owned & hired auto (my aides drive their own cars)?”

“What coverage limits do the NJ FamilyCare MCOs, the VA, or long-term-care insurers require?”

“Can you set up New Jersey workers' comp AND temporary disability coverage?”

PART 4

Get Accredited & Register — Step by Step

In short: Accreditation is a precondition of HCSF registration in New Jersey, so do it first (or in parallel). Then register with Consumer Affairs with your accreditation, bond, and background certifications.

4.1 Get accredited (REQUIRED — start early)

Since 2018, every HCSF must be accredited to register and renew. New Jersey recognizes CHAP, ACHC, and The Joint Commission (plus CAHC and NIHCA). Accreditation means adopting policies and procedures to the accreditor's standards and passing their review — it can take several months, which is exactly why you start it on day one.

- 1. Pick an accreditor and get the standards.** CHAP, ACHC, or TJC are the mainstream choices; request a quote and a standards manual so you know precisely what you're building to.
- 2. Write your policies to the standard.** Client rights, care planning, aide competency, supervision, complaint handling, incident and abuse reporting, emergency preparedness, and infection control — this policy manual also becomes your operating backbone.
- 3. Complete the accreditation review.** Submit your self-study and pass the accreditor's review; then you're eligible to register as an HCSF.

What this means for you: budget several months and \$7,000–\$14,500 for accreditation, and treat it as the gating step your whole launch schedule depends on — everything else (bond, insurance, CHHA hiring) can run in parallel.

4.2 Register as an HCSF with Consumer Affairs

- ☐ Proof of accreditation (CHAP/ACHC/TJC)
- ☐ Proof of the \$10,000 surety bond
- ☐ Criminal-history certifications for officers, directors, principals, and owners (10%+ ownership disclosed)
- ☐ Your managing agent's information
- ☐ Your completed registration application and fee

- 1. File through the DCA Regulated Business portal.** At rgbportal.dca.njoag.gov, with your accreditation, bond, and background certifications attached.
- 2. Pay the registration fee.** Confirm the current HCSF registration and renewal fee directly with Consumer Affairs (973-504-6370) before you rely on a number — third-party sources cite roughly \$500 each.
- 3. Keep it current.** Registration renews on or before July 1 each year; report ownership or management changes within 30 days and file the required financial reporting to DCA.

What this means for you: New Jersey front-loads the work. You can't register — and therefore can't legally operate — until accreditation is done, so your realistic timeline is driven by the accreditor, not the state. The upside is that once you're through it, you're a credentialed, accredited firm that MLTSS plans, the VA, and long-term-care insurers are comfortable contracting with, which shortens every conversation that follows.

PART 5

Your Aides — CHHA Certification

In short: A caregiver who provides hands-on personal care in New Jersey must be a Certified Homemaker–Home Health Aide (CHHA). Build your hiring around the CHHA pipeline and keep meticulous records.

5.1 The CHHA certificate (NJ Board of Nursing) — step by step

1. **Complete an approved training program.** 76 hours total — at least 60 hours classroom plus 16 hours of clinical/practical training — through a Board-approved program.
2. **Pass the fingerprint background check.** A fingerprint-based state and FBI criminal-history check is required before the aide begins employment (New Jersey eliminated temporary/pre-clearance work certificates, so the clearance must be in hand).
3. **Apply online for certification.** Initial CHHA certification is submitted online through the Board of Nursing.
4. **Renew every two years.** Certificates expire November 30 of odd-numbered years; the renewal fee is \$30, and the aide must be actively employed to hold and renew the certificate.

Plan your pipeline. Because hands-on aides must be CHHAs, either hire already-certified CHHAs or partner with a Board-approved training program to build your own. A companion/sitter role with NO hands-on personal care is the exception — but any help with bathing, dressing, or transferring requires a CHHA. Verify each aide's status on the Board's CHHA registry before you assign them.

5.2 Employing & supervising your aides

You employ CHHAs as W-2 employees and handle payroll, workers' comp, disability, and supervision. Keep each aide's CHHA certificate, fingerprint clearance, and training on file, and re-verify certification at renewal — your accreditor and Consumer Affairs both expect current records. Beyond the certificate, invest in orientation, a skills check-off before an aide works alone, and consistent scheduling — sending the same reliable aide every time is the number-one driver of client satisfaction, referrals, and retention.

5.3 Building your CHHA pipeline (the practical playbook)

The hardest ongoing job in New Jersey home care isn't paperwork — it's keeping enough certified aides on your roster to say yes when a discharge planner calls. Because hands-on aides must already be CHHAs, your pipeline is your business. Here is how successful NJ owners keep it full:

1. **Recruit already-certified CHHAs first.** There are tens of thousands of certified CHHAs in New Jersey. Post on Indeed, ZipRecruiter, and CHHA-specific Facebook groups; ask every good hire for referrals (a \$150–\$300 referral bonus pays for itself); and always ask for the aide's certificate number so you can verify it on the Board of Nursing registry before the first shift.
2. **Partner with a Board-approved training school.** Approved CHHA programs run constantly across the state. A standing relationship with one or two schools gives you first look at new graduates — offer to be their placement partner, and some will send you candidates before they've even finished the 76 hours.
3. **Consider becoming an approved training program yourself.** Once you have volume, running your own Board-approved CHHA course turns your biggest cost center into a recruiting funnel — you train aides to your standard and hire the best. It's a bigger commitment, but it's how the largest NJ agencies solve the shortage permanently.
4. **Onboard fast and treat them well.** Aides work for whoever schedules them first and treats them best. A same-week interview, quick fingerprint scheduling, guaranteed hours, and consistent clients beat a higher wage almost every time.

Verify before every assignment. New Jersey holds YOU responsible for confirming an aide's CHHA certificate is active before they touch a client. Make registry verification a hard step in your hiring checklist — an expired or lapsed certificate discovered during an audit can cost you a Medicaid contract.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: *Your accreditation gives you your policy backbone. Add HHAeXchange EVV for Medicaid visits and your scheduling/billing systems, and keep records inspection-ready.*

6.1 Policies (from your accreditation)

Your accreditation process builds most of your policy manual — client rights, care planning and supervision, aide competency, complaint handling, incident and abuse reporting, emergency preparedness, and infection control. Keep it current; it's also what a Medicaid plan's audit reviews. What this means for you: because you had to build these policies to pass accreditation anyway, you start operations with a stronger compliance backbone than agencies in lighter-touch states — use it, don't just file it.

6.2 Set up EVV (before your first Medicaid visit)

1. **Onboard with HHAeXchange.** NJ uses HHAeXchange as the statewide EVV aggregator for Medicaid personal care (some MCOs route through CareBridge into HHAeXchange).
2. **Set up your aides, clients, and authorizations.**
3. **Capture every visit and fix exceptions.** Each visit records who/what/where/when; correct any exceptions so the claim pays.
4. **Bill against verified visits.** A Medicaid claim must match a verified visit — keep your compliance rate high.

Private pay is EVV-free. EVV applies only to Medicaid visits. Launching on private pay means no EVV until your first Medicaid client.

6.3 Systems

Choose a home-care platform for scheduling, records, visit tracking, invoicing, and payroll that integrates with HHAeXchange for your Medicaid line.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via an MCO care manager), you assess and set up the plan, a CHHA delivers the hours (with EVV for Medicaid), you supervise, and you review and adjust. Here is the whole journey, stage by stage.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or an MLTSS care manager (Part 10).

What you do on the first call:

- Take the basics — who needs care, what tasks, how many hours, where, when to start; confirm who pays (private, MLTSS, VA, or long-term-care insurance); and confirm the need is within your scope.

Stage 2 — Assessment & the plan of care

Do an in-home assessment and build a plan of care: the tasks, schedule, and hours, and any safety concerns.

What to review in the home:

- Mobility and transfer needs; fall hazards; medications and who manages them; family and other supports; and, for MLTSS clients, the MCO's authorization, which sets the approved hours your plan must match.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement covering services, schedule, rate/payer, client rights, and how service can end. Assign a certified, registry-verified CHHA and begin with a supervised introduction.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The CHHA delivers care; Medicaid visits are captured in HHAEExchange. Keep visit documentation and protect the client-aide match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

Supervise per your policy and accreditation standard; monitor performance and changing needs; run your quality program (satisfaction, complaints, incident reports); and report any suspected abuse, neglect, or exploitation immediately.

Stage 6 — Changes & reassessment

As needs change, update the plan of care; for MLTSS clients, coordinate reassessments and new authorizations with the MCO care manager. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any MCO, and document the transition. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid (MLTSS), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're registered; add Medicaid by contracting with the NJ FamilyCare plans. New Jersey is a high-cost, high-opportunity market.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately; NJ is high-cost (~mid-\$30s/hr, 2024)
Medicaid — MLTSS / PCA	Managed Long Term Services and Supports + Personal Care	Delivered through the 5 MCOs; requires enrollment, contracts & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 1, Optum)

8.1 Private pay — a strong launch market

The day your registration issues, you serve private-pay clients — families paying out of pocket, typically billed hourly with a visit minimum. New Jersey is one of the highest-cost home-care markets in the country (2024 medians were roughly \$82,000/year for homemaker and \$85,000/year for home health aide services — implying the mid-\$30s per hour; verify the current figure). Set your rate above your fully-loaded (bond-, disability-, and workers'-comp-included) aide cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL or cognitive triggers are met, and typically require a CHHA from an accredited firm — which you are; you bill the family or the insurer. How to set your rate: start from your fully-loaded caregiver cost (wage + payroll taxes + workers' comp + disability + overhead), add your target margin, and check it against what other agencies in your county quote — then set a visit minimum (commonly 3–4 hours) so short visits don't lose money on travel and scheduling. In high-cost northern-NJ counties you can price at the top of the range; price more competitively in lower-cost areas.

8.2 Medicaid — MLTSS & PCA

NJ FamilyCare delivers long-term home care through Managed Long Term Services and Supports (MLTSS) and the state-plan Personal Care Assistant benefit — both through the managed-care organizations. The current plans are Aetna Better Health, Fidelis Care (WellCare), Horizon NJ Health, UnitedHealthcare Community Plan, and Wellpoint (formerly Amerigroup).

- 1. Be a registered, accredited HCSF first (Part 4).**
- 2. Enroll as a NJ Medicaid provider via NJMMIS** to get your Medicaid provider number.
- 3. Contract with each MCO individually.** Credential and negotiate rates plan-by-plan; set up EVV (HHAeXchange).
- 4. Support the Personal Preference Program (optional).** PPP lets a member self-direct and hire their own workers; fiscal intermediaries are PALCO (Horizon) and Public Partnerships/PPL (Aetna, Fidelis, UnitedHealthcare, Wellpoint) — your agency can partner as a supporting role.

How you actually bill (the claims cycle)

- 1. Verify eligibility & the authorization each month.** Confirm the member's plan and the authorized hours before and during care.
- 2. Capture every visit in HHAeXchange (EVV).**
- 3. Submit the claim to the member's MCO.** Using your NPI and the authorized service; the MCO — not the state — pays you.
- 4. Watch timely filing and reconcile remittances.** Confirm each MCO's filing window, and reconcile so nothing goes unpaid.

Medicaid PCA/MLTSS reimbursement is set by the DMAHS fee schedule and each MCO contract and changes periodically — pull the current rate from the schedule or your contract; don't rely on a quoted number.

8.3 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. New Jersey is in Region 1, administered by Optum (provider line 888-901-7407).
- **Aid & Attendance:** an increased VA pension the veteran can apply toward your private-pay care (verify current amounts on VA.gov).

PART 9

What It Costs & How Long It Takes

In short: Budget accreditation (the big one), the bond, insurance, CHHA hiring, EVV, and payroll float. Accreditation is the long pole on the timeline.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
Accreditation (REQUIRED)	CHAP / ACHC / TJC — precondition to register	~\$7,000–\$14,500 / 3-yr cycle
HCSF registration	Consumer Affairs registration + renewal	Confirm with DCA (~\$500 cited)
Surety bond	\$10,000 bond (waivable at \$100k net worth)	~\$100/yr premium
Business setup	Div. of Revenue LLC \$125 + \$75/yr report, NJ-REG, EIN free, NPI free, legal	~\$200 + legal
Insurance	Liability + abuse + non-owned auto + workers' comp + disability	Get quotes — annual
CHHA hiring/training	Certified aides (or a training partnership)	Per aide
EVV & software	HHAeXchange + a scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay aides before Medicaid/insurers pay you	Working capital

Accreditation drives your timeline. Because you can't register without it, accreditation (often a few months) is the critical path. Everything else — bond, insurance, CHHA hiring — can run in parallel.

9.2 Timeline

Month 0	Start accreditation	Pick an accreditor; write policies to the standard
Month 0–1	Form & bond	LLC + NJ-REG + EIN; bind insurance; get the \$10,000 bond
Months 1–4	Complete accreditation	Pass the accreditation review
After accred.	Register the HCSF	File with Consumer Affairs (accreditation + bond + background)
Right away	Start private pay	Serve private-pay & LTC-insurance clients with CHHAs
Then	Contract Medicaid	NJMMIS + the 5 MCOs + HHAeXchange EVV

9.4 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.5 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay aides every week or two, but MLTSS plans and long-term-care insurers pay you 30–60 days after you bill. On a growing Medicaid caseload you can be out tens of thousands of dollars in wages you've paid but not yet collected. Keep 8–12 weeks of payroll in reserve, or launch on private pay (which pays up front) while your Medicaid line ramps.
- **Unbillable overhead:** your own time, your scheduler/coordinator, office and phone, background checks, and the supervisory RN visits your accreditor and MCO contracts may require — none of it is billable, all of it is real.
- **Turnover churn:** every aide who quits costs you re-hiring, re-verifying, and re-training. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a claim that doesn't match a verified EVV visit, or misses timely filing, simply doesn't get paid. Reconcile every remittance and chase exceptions weekly — unreconciled claims are money you earned and lost.
- **Accreditation renewal:** accreditation isn't one-and-done — it runs on a three-year cycle with ongoing fees and a re-survey. Calendar it so it never lapses (a lapse means you can't renew your registration).

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the 21 county Area Agencies on Aging, the ADRC, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the top referral engine — build relationships at NJ systems (RWJBarnabas, Hackensack Meridian, Atlantic Health, Virtua) and make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-skilled hours their benefit doesn't cover; build reciprocal partnerships.
- **Area Agencies on Aging & the ADRC:** New Jersey has 21 county AAAs (operating with the ADRC under the Division of Aging Services) and a statewide ADRC line (1-877-222-3737) — get listed and known.
- **MLTSS care managers:** once contracted, the MCOs' care managers route member hours to in-network agencies.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-/independent-living communities refer supplemental one-on-one care.
- **NJ 2-1-1 & NJ Save:** list with NJ 2-1-1 (dial 211); families using the NJ Save benefits screener often need home-care referrals.

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Registration, accreditation & aides

Who	Contact	What to verify / ask
DCA — Regulated Business Section (HCSF)	(973) 504-6370 · njconsumeraffairs.gov/hcservice	The HCSF registration & fee; the accreditation & bond proof; July 1 renewal; financial reporting
Accreditor — CHAP / ACHC / TJC	chapinc.org · achc.org · jointcommission.org	The required accreditation; cost and timeline
NJ Board of Nursing — CHHA	973-504-6430 · njconsumeraffairs.gov/hhh	CHHA training (76 hrs), certification, renewal, the registry, the fingerprint check
Surety bond broker	a NJ surety agent	The \$10,000 HCSF bond (and the net-worth waiver)

Business, insurance & billing IDs

Who	Contact	What to verify / ask
NJ Division of Revenue & Enterprise Services	nj.gov/treasury/revenue	Forming your LLC (\$125); NJ-REG; the \$75 annual report
IRS	irs.gov (EIN Assistant)	Getting your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI (for Medicaid/VA billing)
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.5	Liability + abuse + non-owned auto + workers' comp + disability; the HCSF bond

Funding, EVV & referrals

Who	Contact	What to verify / ask
NJMMIS — Medicaid provider enrollment	njmmis.com	Enrolling your NPI; getting a Medicaid provider number
NJ FamilyCare MCOs	Aetna, Fidelis, Horizon NJ Health, UnitedHealthcare, Wellpoint	Contracting; MLTSS/PCA rates; authorizations; timely filing; EVV
NJ EVV — HHAExchange	hhaexchange.com	EVV onboarding and which plans route through CareBridge
Personal Preference Program (PPP)	PALCO (Horizon) · PPL (Aetna/Fidelis/UHC/Wellpoint)	The consumer-directed option and your supporting role
VA Community Care / Optum (Region 1)	888-901-7407 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (21 county)	nj.gov/humanservices/doas	Local information & referral for older adults
Aging & Disability Resource Connection	1-877-222-3737	Single-entry access to long-term services & supports
NJ 2-1-1	dial 211 · nj211.org	Free statewide referral line — list your services
NJ Long-Term Care Ombudsman	ltco.nj.gov	Independent resident advocate — give clients the number

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I really have to get accredited before I can even open?

Yes — in New Jersey accreditation is a precondition of HCSF registration, not an optional credential. It's the single biggest thing that determines your launch timeline, so start it on day one and build every policy to your accreditor's standard.

Can I hire aides who aren't CHHAs yet?

For hands-on personal care, no — the aide must hold a current CHHA certificate before providing care, and NJ eliminated temporary work certificates. You can either hire already-certified CHHAs or partner with a Board-approved training program to build your own pipeline; companion-only roles with zero hands-on care are the narrow exception.

You can do this

Opening a New Jersey home care agency takes more upfront work than most states — mandatory accreditation, certified aides, and a bond — but that same bar keeps the market professional and rewards operators who do it right. Take it one rung at a time: start accreditation early, form and bond the business, register as an HCSF, build your CHHA pipeline, launch on private pay, and contract with the NJ FamilyCare plans for Medicaid. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in N.J.S.A. 34:8-43 et seq., N.J.A.C. 13:45B, N.J.S.A. 45:11-24.3, and current NJ FamilyCare information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with the Division of Consumer Affairs, the Board of Nursing, and qualified counsel before you rely on it. Confirm the current HCSF fee and the CHHA requirements before you rely on them.